

● PARENTS HEALTH OS

Parents Health OS

The family health operating system for India.

THARUN GAJULA
FOUNDER

Vision · Why · How
May 2026

SECTION I

The Personal *Why*.

Every Indian son or daughter living away from home carries a quiet weight. This is where the company begins.

Anantapur, and a quiet weight.

I am from **Anantapur, Andhra Pradesh**. My parents live there. Like most Indian parents of their generation, they spent decades putting their children first and their own health last.

No regular checkups. No structured medication routines. No preventive care of any kind. Just a lifetime of *"we will manage."*

And now they face the consequences — the chronic conditions, the declining mobility, the daily difficulties that come from decades of deferred care.

"Sab theek hai."

You call home. Your mother says *everything is fine*. Your father says his knees are "a little stiff, nothing to worry about." You hang up. And the worry begins.

Praan Health, and ₹75,000.

₹75K

for three months of chronic care
management.

I looked for solutions. I found **Praan Health** — a well-funded startup offering chronic care for aging parents.

₹8.5 crore seed from Rainmatter, WEH Ventures, and angels including the PhysicsWallah founders. The approach was promising.

The price was not. *I simply could not afford it. I withdrew my application.*

“If a working professional can't afford this, what about millions of regular Indian families?”

THE QUESTION THAT STARTED PARENTS HEALTH OS

What about the family where the son earns ₹40,000 a month and sends half home? What about the daughter doing everything she can from Bangalore but still cannot get her father in Vijayawada to take his diabetes medication on time?

Nobody is building this version.

WHAT EXISTS

Premium full-service models — Emoha, Anvayaa, Praan Health, Samarth — proving demand at the higher-income urban and NRI end of the market.

Comprehensive plans frequently sit at ₹10K–25K/month or above, out of reach for most regular Indian households.

WHAT DOES NOT EXIST

The version a regular Indian family can afford. The version that costs less than a Jio recharge.

The version that lives on **WhatsApp**, because your 67-year-old parent already knows how to send a voice note. The version that turns a ₹200 lab report into an actionable Telugu explanation.

That is Parents Health OS.

SECTION II

A nation aging *without* a safety net.

Demographics, disease, family structure, and the failure modes of every existing solution.

2023 · TODAY

153M

Indians aged 60+

2050 · PROJECTED

347M

More than doubling in a single generation

By 2050, roughly **1 in 5** Indians will be a senior – over 20% of the population.

India's elderly population by 2050 will exceed the current total population of the United States.

By 2046, elderly will outnumber children aged 0-14 for the first time.

The most vulnerable segment is growing fastest.

80+ POPULATION GROWTH · 2022 → 2050

279%

Predominantly widowed, highly dependent women requiring intensive care infrastructure that does not exist.

DECADAL GROWTH RATE · ELDERLY POPULATION

41%

Vastly outpacing general population growth. India's ageing transition is faster than what high-income countries experienced.

India's seniors are not aging healthily.

66%

of all deaths in India

from non-communicable diseases.

22%

Are **premature deaths** occurring before age 70.

NCDs require daily, sustained, repetitive effort — taking pills on time, monitoring BP, adjusting diet, maintaining mobility. Exactly the kind of effort that has no existing system to support it at an affordable price.

The epidemiological ground truth.

32%

diagnosed with
hypertension

36%

have **measured high BP**
(many undiagnosed)

14%

have **diabetes**

19%

suffer from **bone or joint
disease**

40%+

live with some form of
disability

A major pattern for Indian elders is a triad — **diabetes, hypertension, osteoarthritis.**

DIABETICS IN INDIA

101M

living with diabetes

PRE-DIABETIC POPULATION

136M

on the threshold

One of the most important chronic-care challenges for Indian families – daily, lifelong management at scale.

The joint family — India's only eldercare infrastructure — has weakened sharply.

Today's 30-year-old Indian professional lives in **Bangalore, Hyderabad, Mumbai, Delhi** — or in the United States, Canada, UK, or the Gulf.

Their parents live in **Anantapur, Vijayawada, Nagpur, Coimbatore, Patna**. The geographic distance between parent and child has never been greater, and it is growing every year.

40%+

of India's elderly are in the **poorest wealth quintile**.

18.7%

live **without any income**.

SHARE OF 60+ WHO ARE WOMEN

58%

SHARE OF ELDERLY WOMEN WHO ARE WIDOWS

54%

Statistically more likely to live alone. Statistically more likely to face poverty. Statistically least served by every existing premium eldercare product.

A terrifying black box.

The vacuum left by the weakened joint-family safety net has been filled by **fragmented local agencies** and informal caregivers.

PRICING Opaque	TRAINING Inconsistent	VETTING Minimal	ACCOUNTABILITY None
--------------------------------------	---	---------------------------------------	---

From the family's perspective, choosing an eldercare provider feels like operating blind. The cost of a wrong choice is paid by the parent.

The app download death sentence.

You build a beautiful app. The 70-year-old parent must find it on the Play Store, enter a password, verify an email, navigate 2FA, complete an onboarding tour.

Digital-literacy studies and on-ground experience both show that many Indian seniors struggle with app onboarding, passwords, and multi-step digital flows.

They quietly tell their child *"I do not need it anymore."*

The app is abandoned. The startup blames the user.

The real culprit is the product architecture.

01

Find the app on Play Store

02

Create an account, set a password

03

Verify email, navigate 2FA

04

Complete onboarding tour

RESULT

App is abandoned within a week

The premium pricing wall.

COMPREHENSIVE PLAN PRICING · INCUMBENTS

₹10–25K

per month

Praan Health, Emoha, Anvayaa deliver real value. But the more comprehensive plans become unreachable for many regular Indian families.

The ones where the son sends money home monthly.
The ones where the daughter manages two parents on a single income.

Parents Health OS rejects both failure modes simultaneously.

SECTION III

The market *insight*.

Who pays, who uses, who validates — and the emotional engine that makes the subscription renew.

The buyer is not the user.

The single most important insight in Indian eldercare. Not a philosophical nuance — the architectural foundation of the entire product, pricing, and go-to-market strategy.

THE BUYER

The Child

30 · Hyderabad / Bangalore /
Abroad

Carries chronic guilt and anxiety about their
parent's health. Has disposable income.

Willing to pay for peace of mind.

THE USER

The Parent

65 · Anantapur / Tier 2-3

Resists new apps. Resists premium
spending on themselves. Often downplays
their health challenges to avoid burdening
their children.

THE VALIDATOR

The Doctor

RMP · Curated panel

Validates clinical decisions, signs
prescriptions, sees structured longitudinal
data. Trust transferred from doctor → family.

Parents Health OS sells to the child. It delivers care to the parent.

Geographical Guilt.

In Indian culture, caring for aging parents is not an expectation — it is identity. When the adult child moves cities, the resulting psychological fracture is the most powerful, least price-elastic purchasing driver in the Indian consumer market.

STAGE 01

"Sab theek hai."

"My knees are fine. I am eating well."

The parent habitually downplays their struggles. The child knows they aren't getting the truth, but has no way to verify.

DENIAL

STAGE 02

The "not enough" loop.

"I'm doing everything I can. It's never enough."

Groceries via BigBasket. Medicine via PharmEasy. A weekly call. The mind continuously obsesses over the gaps. No effort ever feels sufficient.

CHRONIC ANXIETY

STAGE 03

Crisis trauma.

"Papa was admitted at 2am."

A single middle-of-the-night hospitalization call makes physical distance agonizing. The spending that follows a crisis is always 10× what prevention would have cost.

ACUTE PAIN

Parents Health OS interrupts at Stage 01.

01 Today's BP reading is visible.

The parent's morning measurement, automatically captured, surfaced on the child's dashboard.

*The guilt loop **breaks**.*

02 Today's medication is confirmed.

Anaya confirmed via WhatsApp that the 8:15 AM pill was taken.

*The anxiety **drops**.*

03 This week's AI summary is delivered.

"Readings stable. No concerns this week." Plain language, sent every Sunday.

*The subscription **renews**.*

Large. Structurally underpenetrated.

DEMAND SIDE

- 153M seniors today, growing to 347M by 2050
- 66% of all deaths from NCDs
- 32% with hypertension
- 14% with diabetes
- 40%+ living with disability

SUPPLY SIDE

Emoha, Anvayaa, Khyaal, Samarth, Praan Health — combined, these players still serve a small fraction of the potential market.

Organized eldercare and senior-living coverage appears **highly underpenetrated** relative to India's senior population.

Demand universal · supply fragmented

SECTION IV

The competitive *landscape.*

Seven players. Real value. One intersection nobody occupies.

The seven players we benchmark against.

PLAYER	STRENGTH	WEAKNESS
Emoha	Brand recognition, emergency infrastructure, companion calls	Not WhatsApp-native AI health intelligence
Anvayaa	Care-manager depth, regional language coverage, dementia services	Operations-heavy; not a scalable software product
Khyaal	Community-first, born on WhatsApp during COVID, large user base	Social engagement focus; not longitudinal health
Samarth	NRI-focused, AI predictive analytics, 350+ cities	Premium positioning, full-service operations-heavy
Praan Health	₹8.5Cr seed from Rainmatter / WEH, strong clinical depth	Premium pricing; significant clinical staffing per family
Practo / mFine	Large doctor networks, lab booking, teleconsult marketplace	Generic, not geriatric-specific, not WhatsApp-native
Medisafe / MyTherapy	Best-in-class medication reminder UX, "Medfriend" alerts	No Indian languages, no AI reports, no WhatsApp

Emoha Elder Care.

POSITIONING

One of India's most recognized senior care brands. Emergency support, companion calls, engagement programming, and comprehensive eldercare plans.

Brand · emergency · companion

STRENGTH

Brand recognition and emergency infrastructure. Tier and pricing has flexed lower for engagement-only options.

WEAKNESS

Not built around a WhatsApp-native AI health-intelligence flow. Comprehensive plans can run far above ₹2,299/month depending on package.

Anvayaa.

POSITIONING

Care-manager model across multiple Indian cities.
Specialized eldercare with regional-language support and dementia services.

Care manager · regional language

Public plan pages describe packages with benefits worth roughly ₹38,000–₹80,000.

STRENGTH

Depth of care management and regional-language coverage. Strong on operations.

WEAKNESS

Care-manager-heavy model means high operating expenditure. Not a scalable software product.
Margin compression with growth.

Khyaal.

POSITIONING

Community-first platform with a large senior-user base. Started as WhatsApp groups during COVID. Launched the Khyaal Card for fraud-safe payments.

Community · WhatsApp-native

STRENGTH

Proved that WhatsApp and community-led interaction can work for Indian seniors. Validated user base at scale.

WEAKNESS

Focuses on social engagement and senior lifestyle. Not structured longitudinal health monitoring. No AI clinical layer.

Samarth.

POSITIONING

NRI-focused eldercare platform. AWS-hosted. Uses AI predictive analytics for engagement-decline detection. Covers 350+ cities.

NRI · AI analytics · 350+ cities

STRENGTH

NRI targeting and tech sophistication. Operating at meaningful geographic spread.

WEAKNESS

Premium positioning. Full-service operations-heavy model. Out of reach for regular Indian families paying in rupees.

Praan Health.

POSITIONING

Lifestyle-first chronic care combining physician-led protocols, clinical nutrition, and strength training. Founded by Navneeth Ramprasad.

₹8.5Cr seed · Rainmatter, WEH, angels

STRENGTH

Strong founding team. Clinical depth. Backed by serious capital. Promising approach.

WEAKNESS

Premium pricing. Operations-heavy. Significant clinical staffing needs per family. **~₹75,000 / 3 months put the founder out — and millions of families with him.**

Teleconsult marketplaces & pill reminders.

PRACTO / MFINE

Teleconsultation marketplaces with lab test booking.

Strength. Large doctor networks.

Weakness. Generic — not geriatric-specific, not WhatsApp-native, English-first, no ongoing health monitoring.

MEDISAFE / MYTHERAPY

Global best-in-class medication reminder apps.

Strength. Excellent UX. Medisafe's "Medfriend" notifies family on missed doses.

Weakness. No Indian language support. No lab report intelligence. No doctor marketplace. No WhatsApp integration.

The intersection nobody occupies.

- 01 **WhatsApp-first** parent interaction — zero friction, no app download.
- 02 **AI lab report analysis** in regional languages — Telugu and Hindi first.
- 03 **Longitudinal tracking** visible to the entire family — child, parent, doctor.
- 04 **Affordable pricing** — ₹500–₹5,000/month, not ₹10,000+.
- 05 **Healthspan focus** — preventive and longitudinal, not just emergency response.

This is the gap. This is the wedge.

SECTION V

The *vision*.

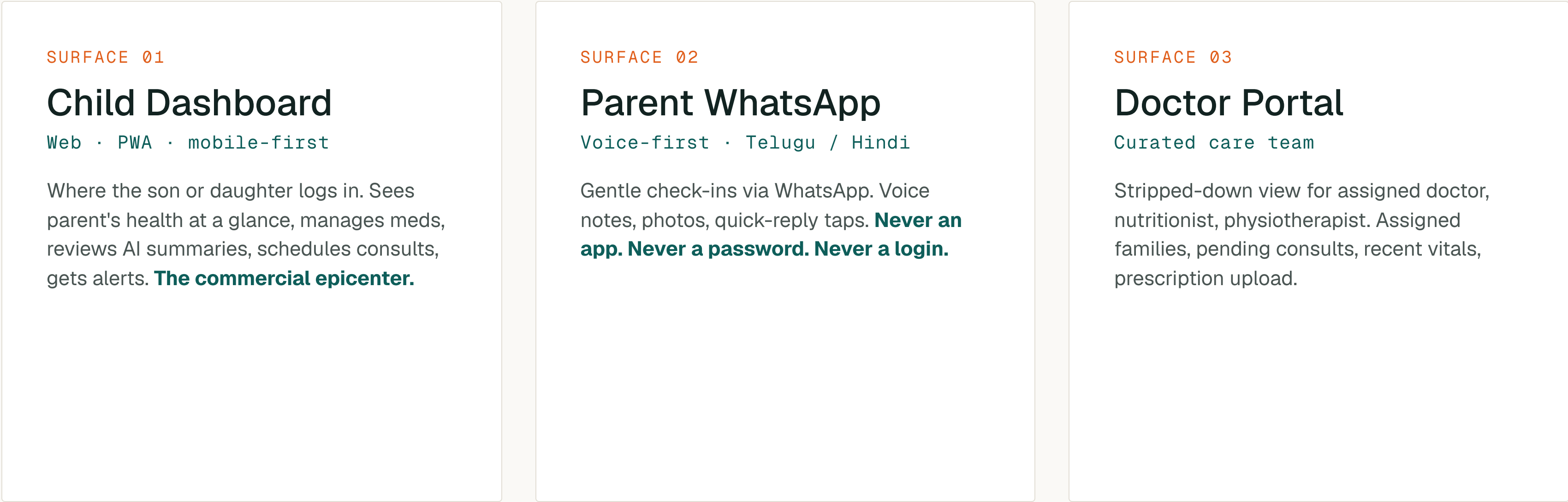
Three surfaces. One database. One minimum viable miracle.

*The family health operating system for India —
where the **child** monitors, **Anaya** reminds, the
parent simply lives, and the **doctor** validates.*

PARENTS HEALTH OS · VISION STATEMENT

Three surfaces, one database.

Parents Health OS is not one product. It is three products sharing one database.



The Child Dashboard.

A responsive Next.js web application, mobile-first, PWA-installable. The son or daughter logs in, sees their parent's health status at a glance, manages medications, reviews AI-generated health summaries, schedules consultations, and receives alerts.

This is the commercial epicenter — the surface the paying customer touches every day.

SURFACE PROFILE

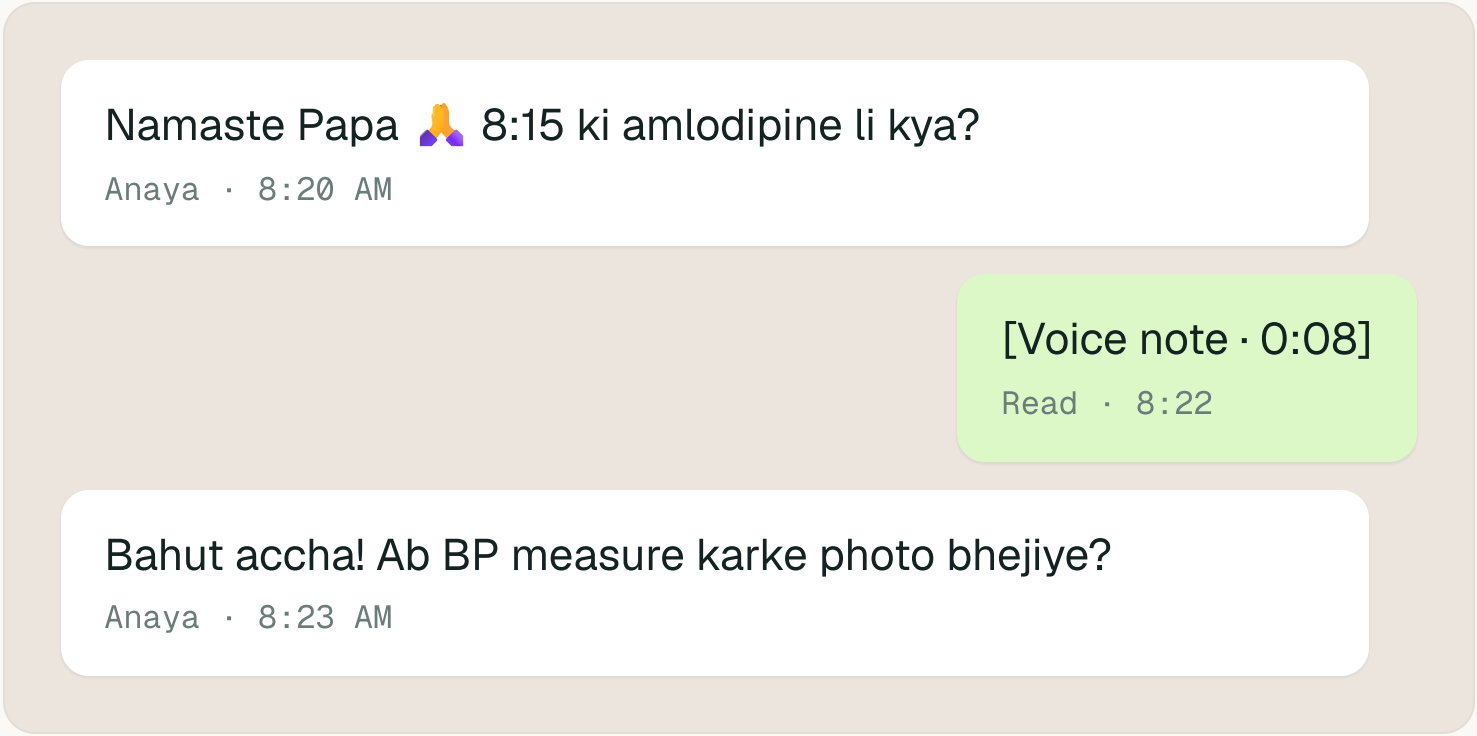
- **Platform:** web, mobile-first, PWA-installable
- **Primary user:** 30 y/o son or daughter
- **Core jobs:** dashboard, meds, alerts, consults
- **Auth:** email magic link
- **Role:** operator

The Parent's WhatsApp.

A real WhatsApp Business Cloud API integration. The parent receives gentle, culturally appropriate check-ins in Telugu or Hindi (with voice notes). They respond by voice, by photo, or by tapping a quick-reply button.

They never download an app. They never create a password. They never see a login screen.

Anaya is positioned as a task-specific health-support workflow for reminders, check-ins, and structured escalation — not a general-purpose AI assistant.



The Doctor Portal.

A stripped-down web view for the assigned doctor, nutritionist, or physiotherapist. They see a list of their assigned families, pending consults, recent vitals, and can upload prescriptions.

Starts as a manually curated panel of 2–3 trusted doctors. Grows into a verified marketplace over time as quality is proven.

SURFACE PROFILE

- **Platform:** web, minimal chrome
- **Primary user:** RMP, nutritionist, physio, counselor
- **Core jobs:** family list, pending consults, vitals, Rx upload
- **Compliance:** NMC registration shown on every Rx
- **Role:** validator

The single killer flow.



This single flow — lab report to Telugu voice note to family-wide visibility — **is the minimum viable miracle.** Everything else is built around it.

*The father heard his lab results **in**
Telugu, in a voice note, inside WhatsApp.*

No app. No login. No friction.

No app download

No password

No login screen

No onboarding tour

No English requirement

SECTION VI

What we *built*.

A live prototype. Seven features. Zero infrastructure cost. Production-deployed today.

The live prototype.

DEPLOYED · LIVE

`parents-health-os.vercel.app`

Production-deployed on Vercel · fully functional

Not a mockup. A working application that demonstrates the complete information flow: parent profiling, daily tracking, AI synthesis, family visibility, care coordination.

STACK

- FE Next.js 16 (App Router)
- UI Tailwind CSS v4 · React 19
- AI Google Gemini 2.5 Flash
- DATA localStorage (prototype-only)
- HOST Vercel · zero infra cost

15-Point Geriatric Health Assessment.

A clinically structured profiling protocol that evaluates a parent across **10 health dimensions** — metabolic, cardiovascular, cognitive, resilience, muscular, frailty, digestive, emotional, sleep, lifestyle.

Generates a composite health index that determines care intensity level. Three-tier classification routes the family to the right level of intervention.

10 HEALTH DIMENSIONS

- Metabolic · Cardiovascular · Cognitive
- Resilience · Muscular · Frailty
- Digestive · Emotional · Sleep · Lifestyle

OUTPUT

- Weighted composite score
- Three-tier classification
- Care intensity recommendation

AI-Powered Lab Report Analysis.

Upload any lab report — PDF, JPEG, PNG. Gemini 2.5 Flash extracts biomarkers, identifies the document type, flags abnormal values, and generates a warm, simple summary.

If a prescription is uploaded, the AI can suggest medication entries — but a human user or clinician **must explicitly confirm** them before they become active.

INPUT TYPES

- Lab report PDF
- Photo of printed report (JPEG/PNG)
- Prescription scan

AI PIPELINE

- Extract biomarkers · classify document
- Flag abnormalities · ranges
- Plain-language summary · medication suggestions

Daily Care Dashboard.

A unified status center showing medication compliance rates, current vital statistics, observation feeds, and scheduled checkups.

Medications grouped by time slot — **Morning, Noon, Evening, Night** — with live compliance tracking.

SURFACES TODAY

- Medication compliance · by time slot
- Current vitals · BP, glucose, weight
- Observation feed · qualitative notes
- Scheduled checkups · upcoming appointments

Vitals Logging System.

Quick-entry forms for blood pressure, blood glucose, weight, and general health observations. A history matrix provides at-a-glance compliance visibility.

30-day color matrix: **green** for fully logged · **amber** for partial · **red** for nothing logged.

TRACKED VITALS

- Blood pressure
- Blood glucose
- Weight
- General observations (qualitative)

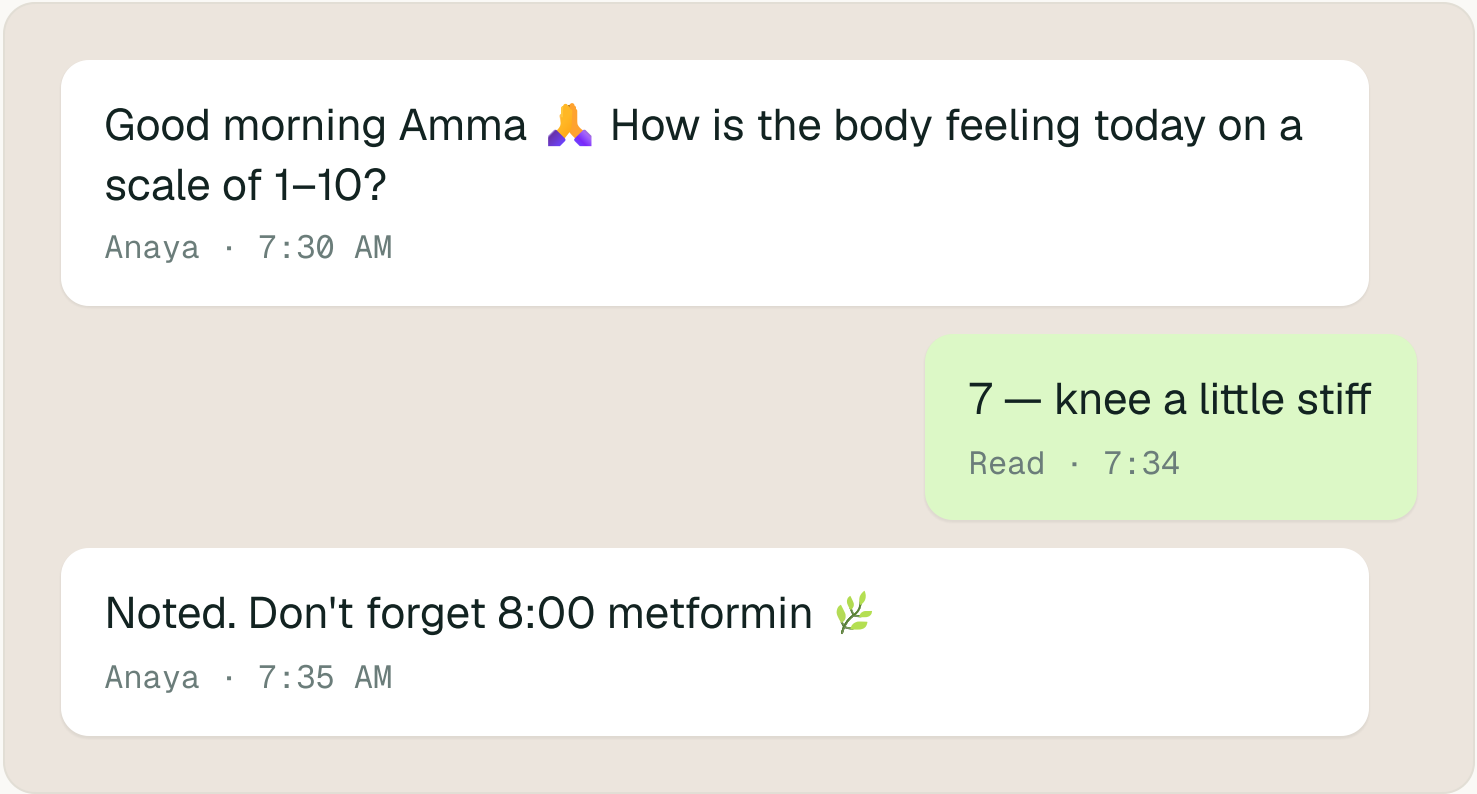
VISIBILITY

- 30-day matrix · green / amber / red
- Trend lines · weekly + monthly

Anaya — Simulated WhatsApp Workflow.

A demonstration of the conversational interface. Anaya confirms medication adherence, requests pain-scale assessments, provides supportive feedback, and simulates daily routine synchronization.

Currently simulated. The roadmap moves this to the real WhatsApp Business Cloud API.



Profile System.

Support for multiple parent profiles — Amma / Papa switching. Portable JSON backup and restore.

Consent-gated AI features with explicit privacy disclaimers built into the flow.

CAPABILITIES

- Multi-parent profiles (Amma / Papa)
- Portable JSON backup / restore
- Consent gates on every AI call
- Explicit privacy disclaimers in-flow

Consultation Brief Generator.

A printable clinical summary that aggregates profile data, recent vitals, active medications, and AI insights into a formatted document.

Suitable for handing to a doctor during a physical visit — bridges the digital platform with the still-paper-based world of small-town Indian healthcare.

AGGREGATES

- Profile summary + assessment
- Recent vitals (30 days)
- Active medications
- AI insights · flagged concerns

Two propositions, validated.

PROPOSITION 01

Technically feasible on free-tier infrastructure.

The entire application runs on Vercel (free), uses Gemini Flash (free tier), and stores data locally. **Zero infrastructure cost** at prototype scale.

PROPOSITION 02

Buildable by a single developer.

A functional health coordination platform — in weeks, not months. The AI leverage is real. What once required a clinical data team now requires **one developer and an API key**.

SECTION VII

From prototype to *production.*

Three transitions. Four phases. One hundred families.

From sandbox to system.

TRANSITION 01

Storage

localStorage → Supabase

PostgreSQL hosted in **ap-south-1 Mumbai**. Multi-device access, family sharing, audit logging, row-level security. Better latency, trust, and risk posture.

TRANSITION 02

Messaging

Simulated → WhatsApp Cloud API

Real Meta webhooks, template approvals, voice note processing via Gemini, interactive quick-reply buttons. **The single most important transition.**

TRANSITION 03

Architecture

Solo app → Family OS

Distinct roles — child as operator, parent as subject, doctor as validator. Separate auth flows. Role-based dashboards.

Four phases to the first hundred families.

Phase 01 · FOUNDATION

Lock infrastructure.

Supabase project (ap-south-1), authentication (email + phone OTP), database schema with RLS, Vercel deployment with env vars.

Phase 02 · CORE

Build child dashboard + WhatsApp + AI.

Parent overview, vitals timeline, medication management, alert system. Wire Cloud API. Build Gemini pipeline.

Phase 03 · FAMILY

Family member + doctor portal.

Invite siblings, assign roles. Build doctor portal. Progressive Katz ADL + GDS-15 delivered via WhatsApp across days.

Phase 04 · BETA

10 → 100 families.

Founder does every onboarding personally. Fix everything that breaks. Expand from network.

100

families. Every onboarding done personally by the founder.

PHASE	ACQUISITION	MARKETING SPEND
Concierge	Founder's personal network	₹0

SECTION VIII

The operating *model*.

Where margins come from. Where trust comes from. Where they meet.

Where margins come from.

Entirely software and AI. This is where the asymmetric leverage lives.

COMPONENT	VENDOR
Messaging	WhatsApp Cloud API
AI · transcription · translation · analysis	Gemini 2.5 Flash
Storage · auth · realtime	Supabase (ap-south-1)
Frontend + API layer	Next.js on Vercel

INFRASTRUCTURE COST AT 100 FAMILIES

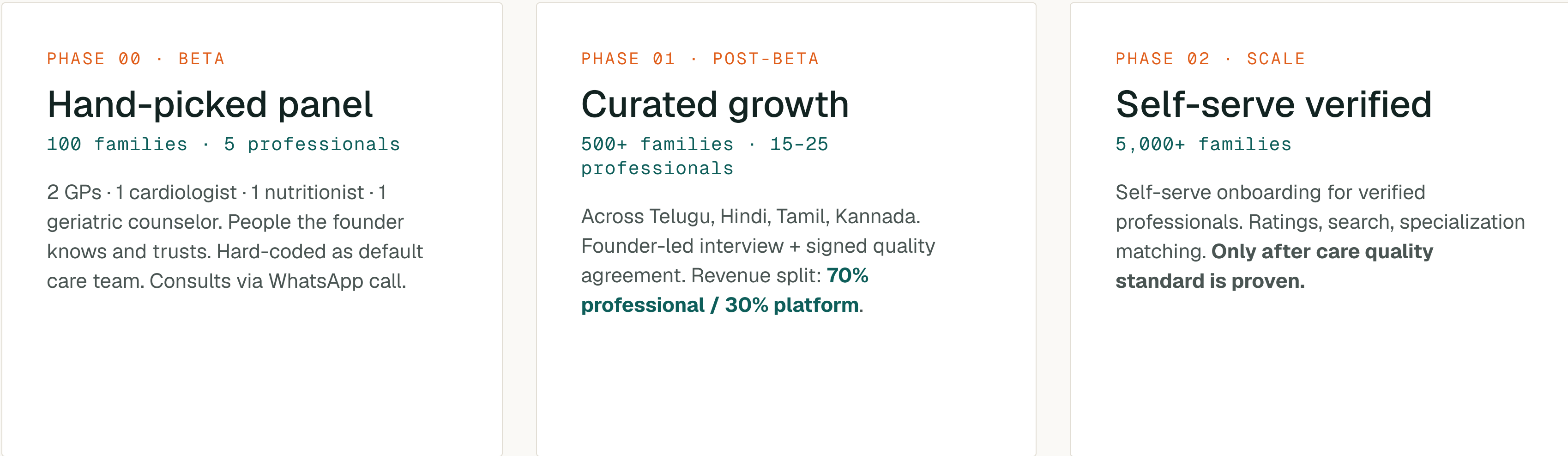
₹3–5K

per month, ex-messaging

WhatsApp is charged per delivered conversation; the financial model should use the latest Meta India rate card at launch.

Where trust comes from.

Technology alone does not deliver healthcare. Parents Health OS operates with a curated care team model that grows in three phases.



The bar to join the panel.

DOCTORS

- REQ NMC or State Medical Council registration
- REQ Registration displayed on every prescription and message
- REQ Founder-led interview
- REQ Signed quality agreement
- REQ Background verification

ALLIED PROFESSIONALS

- Nutritionists · physiotherapists · counselors
- REQ Relevant qualification
 - REQ License or registration where applicable
 - REQ Identity verification
 - REQ Background verification
 - REQ Founder-led interview + quality agreement

Same schema. Replaced progressively.

Every professional category in the platform – doctors, nutritionists, physiotherapists, counselors – follows the same pattern.

Start with dummy data to build the interface.

Validate the user flow with real beta families. Replace dummy data with real professionals, one category at a time.

The database schema is designed from day one to accommodate real profiles. Same tables tomorrow as today – no migrations required.

DAY 1

Schema with dummy professional profiles

WEEK 1

UI validated with real beta families

WEEK 4

First real doctor swapped in

MONTH 3

All professional categories live

RESULT

Zero schema migration. Zero rebuilt UI.

Telemedicine Practice Guidelines 2020.

Notified by the Ministry of Health and Family Welfare. Governs every teleconsultation on the platform.

REQUIREMENT	HOW PARENTS HEALTH OS COMPLIES
Doctor must be RMP	Valid NMC / State Medical Council number, displayed on every prescription and WhatsApp message.
Consent rules	Implied if patient initiates. Explicit consent recorded if platform initiates (e.g., "Would you like to talk to Dr. Rao about your BP trend?").
Schedule X restrictions	Schedule X drugs and any Narcotic Drugs and Psychotropic Substances cannot be prescribed via teleconsultation.
Record retention	All consultation records retained for a minimum of 3 years .

SECTION IX

The business *model*.

Pricing philosophy. Three tiers. Unit economics. The path to one crore ARR.

*Preventive parent care should cost **less** than what families already spend on **reactive** healthcare.*

THE ANCHORING PRINCIPLE

Emergency doctor visits. Hospital stays. Pharmacy runs for symptoms that could have been prevented. The everyday cost of "we will manage" — re-spent every single month.

Pricing, anchored to the Indian family.

CARE LITE

Foundation

₹599 / month

1 parent

- WhatsApp check-ins
- Dashboard access
- AI lab report analysis
- No doctor consults

CARE PLUS

Most chosen

₹1,999 / month

1 parent

- Everything in Lite
- 2 doctor consults / month
- Nutritionist access
- Priority alerts

FAMILY PLAN

For both parents

₹2,999 / month

2 parents

- Everything in Plus for both
- 4 total consults / month
- Dedicated care coordinator

Annual prepay · 30% off any plan

The beta math.

LINE ITEM	₹ / MONTH
Revenue (100 × ₹1,500 ARPU)	1,50,000
Supabase Pro	2,100
Vercel Pro	1,680
Gemini Tier 1 (paid)	1,000–2,000
WhatsApp messaging	variable
Domain + email	500
Doctor payouts (2 × ₹300/family)	60,000
Founder salary (bootstrapped)	0
Total costs	~68,000–70,000

GROSS MARGIN AT 100 FAMILIES

~50–55%

Improves **dramatically at scale** — infrastructure costs are largely fixed (Supabase, Vercel, Gemini scale sub-linearly) while revenue scales linearly with families.

₹1 Crore ARR = ₹8.33L MRR.

AVERAGE ARPU	FAMILIES NEEDED	ACHIEVABLE FROM
₹3,000	278 families	A single neighborhood
₹2,000	417 families	One city cluster
₹1,500	556 families	One community network

Not a national launch. Not a million-dollar marketing budget. 300–500 families who trust the product enough to pay for it monthly.

SECTION X

Privacy & *compliance*.

DPDPA 2023. Seven day-one requirements. Why this is a competitive moat.

The Digital Personal Data Protection Act.

India's **Digital Personal Data Protection Act (2023)**, with operational rules notified in November 2025, applies to digital personal data — *including* health-related personal data.

The Act does not create a separate legal category called "sensitive personal data." Parents Health OS still treats health data as **operationally high-risk** and applies stricter internal protections.

OUR ROLE

Data Fiduciary.

The entity that determines the purpose and means of processing personal health data — and is accountable for every consent, every transfer, every retention decision.

Built in from beta, not bolted on later.

01

Privacy notice (EN / HI / TE)

Itemized list of every data category, purpose, and withdrawal mechanism.

02

Granular consent

Separate opt-ins per purpose. Never bundled into one checkbox.

03

Consent ledger

Every action stored with timestamp, locale, version, IP, evidence.

04

Withdrawal mechanism

"STOP" on WhatsApp · "Delete my data" on dashboard. One tap.

05

Grievance officer

Named person, email address, 90-day response SLA.

06

Data location transparency

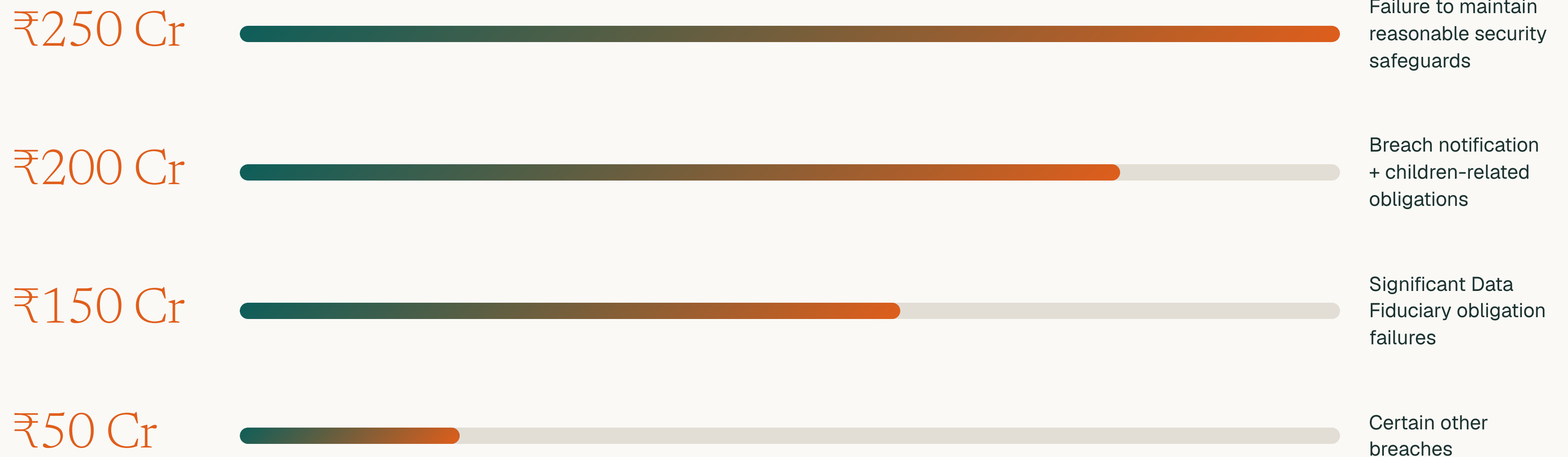
Primary production data in India. Cross-border processors disclosed.

07

Right to access, correct, erase

"Download my data" button on the dashboard. One-tap deletion workflow.

DPDPA maximum penalties.



Designing for compliance from the start is both the legal and the ethical obligation – especially when the data involves the health of vulnerable elderly citizens.

The reason enterprise says yes.

Many fragmented eldercare providers operate without a mature digital data-protection framework.

The moment enterprise customers — insurance companies, corporates offering elder-care benefits — evaluate partners, they run every vendor past their **risk, legal, and procurement teams.**

A provider that cannot demonstrate privacy and security controls is far less likely to pass diligence.

We pass diligence on day one.

Not a regulatory checkbox. The **reason an enterprise says yes** when their risk team says no to every competitor.

SECTION XI

The go-to-market *plan.*

From the founder's hundred to community clusters to B2B2C distribution.

The Founder's 100.

The first 100 families come from the founder's personal network. Relatives, friends, colleagues, extended community.

This is the concierge phase — every onboarding done personally, every bug fixed in real-time, every parent called to verify they understand the WhatsApp interaction.

No paid marketing. No social campaigns. **Just WhatsApp forwards and personal conversations.**

THE PITCH SCRIPT

"I built this for my own parents. It helps you see your parent's health status every day, reminds them to take their medicine, and explains their lab reports in Telugu.

Try it free for one month. If it helps, stay. If not, delete everything with one tap."

Community clusters.

After 100, growth comes from community-level activation — densely-clustered trust networks where one endorsement converts many families.

CHANNEL 01

RWAs

Resident Welfare Associations

Affluent Indian seniors concentrate in gated communities. A weekend health awareness session + free assessments can convert an entire apartment complex. Same doctor serves all families efficiently.

CHANNEL 02

Religious & cultural orgs

Temples · churches · community
associations

Trusted institutions in Indian society. A single endorsement from a respected community leader converts families that no Instagram ad ever could.

CHANNEL 03

Word of mouth

Sibling → cousin → colleague

One satisfied customer tells their siblings, who tell their friends. The referral chain in Indian eldercare is uniquely powerful — the product addresses a universally felt emotional pain.

The B2B2C distribution pivot.

At 500+ families, individual CAC begins to rise. The sustainable growth path is institutional distribution.

CHANNEL 01 · CORPORATE BENEFITS

Eldercare logistics = productivity drain.

India's major employers know mid-career professionals are losing productivity to eldercare logistics. A working mother spending 45 minutes every morning coordinating her father's medication in a different city is not fully present at work.

Sold as a retention strategy, not a healthcare perk.

CHANNEL 02 · INSURANCE

Reduce avoidable hospitalizations.

Indian health insurers face high incurred claim ratios on senior policies. Better monitoring may help reduce avoidable complications — a fall risk noticed early, a cardiac risk escalated because BP was tracked.

Offered as a preventive benefit to premium senior policyholders.

SECTION XII

What we will *never*
build.

The product's negative space. The lines we will not cross.

NO · 01

No streaks that punish missed days.

Health is not a game. A parent who misses a day sees a gentle rhythm view that celebrates patterns — not a broken streak that induces guilt.

NO · 02

No calorie or macro tracking.

Asking a 67-year-old Telugu mother to weigh her sambar is absurd. The system tracks qualitative patterns — "ate well today," "skipped lunch" — through simple WhatsApp interactions.

NO · 03

No public leaderboards.

Health is private. Family health is sacred. There will never be a feature where one family's compliance is compared to another's.

NO · 04

No medical diagnoses or prescriptions.

Anaya is an assistant, not a doctor. Every AI output is informational, not clinical. Every concerning pattern is escalated to a real human RMP.

SECTION XIII

The north *star*.

One metric that cannot be gamed by one side alone.

WFHT — the only metric we report.

DEFINITION

In any seven-day window, the count of **parent health data points** (a BP reading, a medication confirmation, a voice note check-in) that are **paired with a child or family member acknowledgment** (viewing the dashboard, sending a warm message, acknowledging an alert).

BETA HYPOTHESIS

5+

WFHTs per family per week

within the first 30 days as an early retention signal. To be validated during beta.

- 01 Captures the value proposition.
The parent is sharing. The family is engaged.
- 02 Cannot be gamed by one side alone.
Requires both signals to count.
- 03 Predicts retention.
Parent without an audience stops. Child without fresh data churns.

SECTION XIV

What we *need*.

Three asks. Each unlocks a year of velocity.

01

A clinical co-founder or advisor.

MD or MPH background. Validates assessment protocols, reviews AI outputs, establishes clinical SOPs, builds credibility with doctors joining the care team.

02

A WhatsApp / conversational AI specialist.

The WhatsApp bot is the most technically complex and highest-value component. Experience building production WhatsApp Business API integrations would accelerate the timeline dramatically.

03

Seed capital — ₹5–10 lakhs.

Six months of paid infrastructure: Supabase Pro, Vercel Pro, Gemini paid tier, WhatsApp messaging, domain, dedicated phone number for the WhatsApp Business API.

CLOSING

The *conviction*.

Why this company exists. Why it has to exist now. Why it has to be us.

India has **153 million seniors** today and will have **347 million** by 2050. Two-thirds of them live with chronic diseases. Their children are scattered across cities and continents, carrying guilt and anxiety that no amount of phone calls can resolve.

The existing solutions either cost too much, require too much from the parent, or both.

Parents Health OS is the answer that should have existed a decade ago — a WhatsApp-native, AI-powered, family-shared health operating system that costs less than what most Indian families spend on their monthly mobile recharge.

Prototype is live.

Technology works.

First 100 families in the founder's contacts.

PARENTS HEALTH OS · MAY 2026

Built for the parents of India.
By a son who saw the gap and decided to **fill it.**

THARUN GAJULA
Founder, Parents Health OS

parents-health-os.vercel.app
Anantapur → India → the world